

AFib Anticoagulation Restart Timing After Acute Ischemic Stroke

Evidence-based schedule for starting or restarting DOACs after acute ischemic stroke or TIA.

CLINICAL EFFICACY & SAFETY

Early DOAC initiation in patients with AFib and recent stroke is safe and reasonable (Class I, LOE A). It does not increase the risk of symptomatic intracranial hemorrhage (sICH) compared to delayed initiation, and reduces recurrent ischemic stroke. DOACs are preferred over warfarin.

1. Stroke Severity Classification (ELAN Criteria)

Mild Stroke (NIHSS < 8)

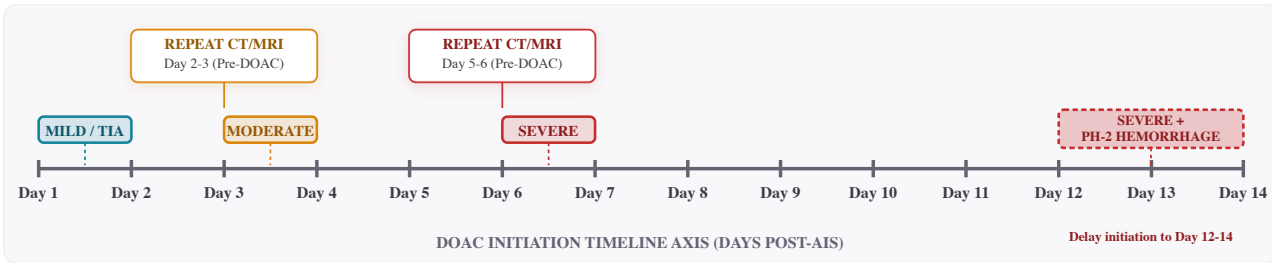
- TIA or infarct < 1.5 cm on brain imaging (cortical or deep subcortical).

Moderate (NIHSS 8–15)

- Occlusion of a **single superficial branch** of the MCA, PCA, or ACA, or isolated brainstem/cerebellar lesion.

Severe (NIHSS ≥ 16)

- Multilobar** infarct, major main stem occlusion (e.g. M1 MCA, P1 PCA, A1 ACA), or large brainstem/cerebellar lesion.



2. Bedside DOAC Dosing & Adjustment Guide

Drug	Standard Dose	Dose Reduction Criteria
Apixaban (Eliquis)	5 mg BID	Reduce to 2.5 mg BID if ≥ 2 criteria are met: • Age ≥ 80 years • Weight ≤ 60 kg • Serum creatinine ≥ 1.5 mg/dL
Rivaroxaban (Xarelto)	20 mg daily (with food)	Reduce to 15 mg daily if CrCl is 15–50 mL/min. Hold if CrCl < 15 mL/min.
Dabigatran (Pradaxa)	150 mg BID	Reduce to 75 mg BID if CrCl is 15–30 mL/min. Avoid if CrCl < 15 mL/min.
Edoxaban (Savaysa)	60 mg daily	Reduce to 30 mg daily if CrCl is 15–50 mL/min or weight ≤ 60 kg. Avoid if CrCl > 95 mL/min (high renal clearance reduces DOAC level).

ELAN Trial: Fischer U et al. *N Engl J Med.* 2023;388:2411–2421. | **CATALYST Meta-Analysis:** Fischer U et al. *Lancet* 2025. Early DOAC (median Day 2) is non-inferior to delayed (median Day 7–8).

AFib Guidelines: Joglar JA et al. 2023 ACC/AHA/ACCP/HRS Guideline. *Circulation.* 2024;149:e1–e156.